



PARTNERSHIP BRIEF

Connecting communities to better care.

A community health platform designed to help health workers recognize needs, connect people to support and follow up on their care.

AFYA OS brings together mental health, everyday physical healthcare and ongoing support for noncommunicable diseases such as diabetes and hypertension. The aim is to make the journey from a community visit to appropriate care more consistent and easier to follow.

Our vision

A person's need for care should not be lost between a household visit, a referral and the next follow-up.

We are seeking partners in Kenya to help shape a practical, community-based programme: health-service providers, county health teams, community organizations, research partners and funders.

Current stage: AFYA OS is an early-stage, pre-pilot project. This brief describes the intended offering. Operational deployment, service partnerships and health benefits remain to be demonstrated.

01 / WHAT AFYA OS AIMS TO DO

Support that follows the person

Community health workers often help people take the first step toward care. AFYA OS is being developed to support that work and help participating services see what happens next.

Area of care	Intended support
Mental health and wellbeing	Respectful conversations, approved assessments, connection to qualified support and follow-up. A proposed initial focus is underserved young people.
Primary healthcare	Help workers record concerns, use approved guidance and coordinate referrals within their professional role.
Long-term conditions	Support follow-up, care-plan tasks and referrals for people with conditions such as hypertension, diabetes and chronic respiratory illness.

A simple care journey

Listen and understand. A community health worker discusses the person’s concerns and explains the proposed support.

Connect to appropriate care. Where needed, the worker helps arrange a referral or other agreed support.

Follow up. The participating team checks whether the person reached the service and what further help is needed.

Learn and improve. Partners review whether the programme is reaching people and making a meaningful difference.

Human care stays central. AFYA OS is designed to support health workers and qualified professionals. It does not replace clinical judgment, make autonomous diagnoses or prescribe treatment.

02 / SHARED VALUE

Why partner with AFYA OS?

The goal is a programme that works for people receiving care and for the organizations serving them. Partner knowledge, local trust and service capacity are essential to making that possible.

Potential partner	Opportunity to contribute
County health teams	Help identify local priorities, suitable communities and a practical connection to existing services.
Healthcare and mental health providers	Shape the support pathway, provide qualified care and supervision, and define appropriate referrals.
Community organizations	Bring local understanding, build trust and include community voices in programme design.
Researchers and evaluators	Help define meaningful measures and assess feasibility, experience and results independently.
Funders and development partners	Support programme design and, where agreed, a properly resourced pilot and evaluation.

What we want to learn together

Can people reach appropriate support more easily? Are referrals followed through? Is follow-up manageable for workers? Do participants find the service respectful and useful? Where suitable, do approved measures show improvement in wellbeing or care continuity? These are questions to test, not results already achieved. Any funding linked to results would require an agreed definition of success and independent verification.

Community participation matters. People receiving care, including young people where relevant, should help shape the programme’s language, accessibility, confidentiality and feedback arrangements.

03 / PARTNERSHIP DISCUSSION

Let’s shape a practical first step

We propose starting with a focused local programme, sized around partner readiness and available care capacity. The location, participants, responsibilities, costs and timeline would be agreed together.

Step	What we agree
1. Explore the fit	Discuss the community need, your organization’s priorities and where collaboration could add value.
2. Define the programme	Agree who will be served, the available support, referral arrangements and each partner’s responsibilities.
3. Prepare responsibly	Confirm qualified oversight, participant protection, permissions, resources and a realistic evaluation plan.
4. Test and review	Proceed with a limited pilot only when ready, then use evidence and participant feedback to decide what comes next.

Commitments that guide the work

Respect and privacy: collect and share only what is appropriate for the agreed purpose.

Qualified oversight: use approved care content and clear safeguarding responsibilities.

Honest evidence: distinguish planned benefits from observed results.

Local fit: work with existing services and the people who use them.

Start a conversation

Tell us the population you serve, the care gap you want to address and the contribution your organization could make. An initial discussion does not require a funding commitment.

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